

USAID Dismantlement and the Global Aid Vacuum

Sector: Social Impact & Global Development | Research Brief | Prepared September 2026

1. Key Findings

A. Multilateral Health Financing Collapse

Global development assistance for health fell 21% between 2024 and 2025, driven almost entirely by a 67% drop in U.S. financing (over \$9 billion), according to the Institute for Health Metrics and Evaluation. The root cause is structural: roughly 5,341 USAID grants and contracts worth \$75.9 billion were terminated following the January 2025 executive orders exiting the WHO and freezing USAID funds, and the agency was formally dissolved shortly after, eliminating decades of bilateral disease-surveillance and delivery infrastructure almost overnight rather than through a managed transition.

B. Multilateral Replenishment Shortfalls

The two flagship multilateral vehicles built to absorb bilateral shocks are themselves undersubscribed: the Global Fund had secured only about \$11 billion of its \$18 billion target for 2026-2028 as of late 2025, and Gavi's June 2025 summit raised over \$9 billion but remained \$2.9 billion short of its \$11.9 billion goal. Consequences are concrete and measurable: Gavi warns the shortfall could leave 75 million children missing routine vaccinations over five years, resulting in an estimated 1.2 million additional child deaths, while the Global Fund's 2024-2026 TB/HIV/malaria grant cycle has already been cut by \$1.4 billion (11%).

C. A Fragmented, Partial Geopolitical Replacement

China, Gulf states, and European bilateral donors (Germany, Denmark, Sweden, Ireland) have made targeted pledges, but analysts at the Lowy Institute and Council on Strategic Risks agree these are opportunistic and regionally selective rather than a systemic replacement, leaving an estimated \$54+ billion aid shortfall largely unaddressed. China's own financing has been in relative decline since 2017 due to debt-sustainability concerns, and experts note it has 'no strategic interest in fully filling the void,' engaging mainly where soft-power returns are highest (e.g., disaster response in the Pacific and Southeast Asia).

D. Philanthropy Is Surging But Cannot Match Scale

Private megadonors have sharply increased giving — MacKenzie Scott gave \$7.1 billion in 2025 (versus \$2.6 billion in 2024), including a twelve-fold increase to organizations in low- and middle-income countries (\$1.2 billion) — and U.S. total philanthropy reached \$617.2 billion in 2025. Even so, this remains a small fraction of the \$54+ billion bilateral shortfall, is concentrated among a handful of donors, and lacks the epidemiological surveillance, logistics, and health-system-strengthening functions that USAID historically performed.

2. Relevant Trends & Statistics (2023-2026)

Indicator	Figure	Source / Year
USAID grants/contracts terminated	5,341 contracts; \$75.9B total value	NYT data, 2025
Global health aid decline (2024→2025)	-21% overall; -67% U.S. share (>\$9B)	IHME, 2025
Projected excess deaths by 2030	9.4M (baseline cuts) / 22.6M (deepened cuts)	Lancet / ISGlobal, 2026
Estimated lives lost in 2025 alone	500,000-1,000,000	Center for Global Dev., 2025
Global Fund 8th replenishment (2026-28)	~\$11B secured vs. \$18B target (61%)	Global Fund, late 2025
Gavi 2025 summit pledge vs. target	\$9B raised vs. \$11.9B target (-\$2.9B gap)	Gavi, Jun 2025
Children at risk from Gavi shortfall	75M miss routine vaccines; ~1.2M child deaths	Gavi / WHS, 2025
TB/HIV/malaria grant cycle cut	-\$1.4B (-11%) of 2024-26 allocation	WHO Global TB Report, 2025
Aid shortfall filled by other actors	\$54B+ shortfall vs. partial pledges	CSR / Lowy Inst., 2025
MacKenzie Scott giving, 2025	\$7.1B total; \$1.2B to LMIC organizations (12x)	Devex / AP, 2025
Total U.S. philanthropy, 2025	\$617.2B (+5.7% YoY)	Giving USA / Lilly School, 2026

3. Important Insights

Emerging Strategies & Innovations

- Unrestricted, regrantor-routed philanthropy:** Scott's model directs funds through intermediary regranteeing funds (nearly 80 of her 2025 recipients) to reach local/national organizations faster than traditional bilateral grant cycles allow, bypassing compliance bottlenecks that block smaller NGOs from large institutional funding.

- **Regional self-reliance and manufacturing localization:** The African Union is accelerating vaccine manufacturing self-sufficiency to reduce dependence on Gavi/USAID supply chains, a structural (not just financial) response to donor volatility.
- **Multilateral consolidation proposals:** Global health leaders (WHO, Gavi, Global Fund) are publicly debating merging overlapping agencies and shifting from periodic 'replenishment' cycles to more resilient, diversified financing models, acknowledging the current system is 'not sustainable.'
- **Digital/AI-enabled efficiency tools:** NGOs and health ministries are increasingly piloting predictive supply-chain analytics and AI-assisted case triage/data systems to preserve service coverage despite fewer field staff and shrinking bilateral technical assistance.
- **Co-financing transition acceleration:** Gavi's model, which gradually shifts cost-share to recipient governments, is being reconsidered for faster (rather than gradual) transition to reduce single-donor exposure.

Key Implementation Bottlenecks

- **Speed mismatch:** Bilateral funding was cut abruptly (weeks), while multilateral replenishment and philanthropic scale-up operate on 3-5 year cycles — a timing gap that field programs cannot bridge.
- **Absorptive-capacity gap:** Local/national NGOs often lack the financial-reporting and compliance infrastructure that new funders (multilateral or corporate philanthropic) require, limiting how quickly displaced funding can be redirected to the organizations actually delivering services.
- **Donor fragmentation and divergent terms:** China, Gulf states, and private philanthropy each bring different reporting requirements, strategic conditions, and debt structures (e.g., Belt and Road loans vs. USAID grants), raising coordination costs and reducing predictability for recipient governments.
- **Scale mismatch:** Even record-setting philanthropic totals (\$7-8B from top individual donors, \$617B sector-wide) remain a fraction of the \$54B+ shortfall and cannot replace government-to-government health system financing.
- **Geopolitical reluctance:** Analysts across the political spectrum agree China has 'no strategic interest' in fully replacing USAID, meaning some gaps — particularly in fragile states with low strategic value — may simply go unfilled.

4. Simple Summary (Executive Brief)

The 2025 dissolution of USAID and U.S. withdrawal from the WHO removed a funding pillar responsible for roughly half of global TB financing and a significant share of vaccine and HIV/AIDS programming, with modeling projecting up to 9.4-22.6 million excess deaths by 2030 if the gap persists. No single alternative architecture is absorbing the loss: multilateral funds (Global Fund, Gavi) remain billions short of their own targets, geopolitical rivals like China are filling only selective, strategically convenient gaps, and record philanthropic giving (\$7.1B from MacKenzie Scott alone) covers only a small fraction of the estimated \$54+ billion shortfall. The irreplaceable gap is not primarily financial but structural — decades of bilateral disease-surveillance, logistics, and local-partner infrastructure that no combination of current alternative funders is positioned to rebuild at comparable speed or scale.

References

- CNN, "One year on from dismantling of USAID, study projects that global aid cuts could lead to 9.4 million deaths by 2030" (2026) — cnn.com/2026/02/04/world/lancet-usaid-global-aid-cuts-intl
- Global Policy Journal, "Consequences and Implications for the International Development Assistance Sector from the Closure of USAID" (2025) — globalpolicyjournal.com
- DevelopmentAid, "U.S. slashing Gavi vaccine funding: 75 million children at risk" (2025) — developmentaid.org
- Health Policy Watch, "Global Health Leaders Urge Fewer Agencies Amid Funding Crisis" (2025) — healthpolicy-watch.news
- Forbes, "After USAID And WHO: Global Health Without The U.S." (2026) — forbes.com/sites/sylvanaqsinha
- WHO, "Impact of 2025 funding cuts" — Global Tuberculosis Report 2025 — who.int/teams/global-programme-on-tuberculosis-and-lung-health
- ORF, "The Potential Impact of Proposed Gavi Funding Cuts on Global Health Security" (2025) — orfonline.org
- Global Biodefense, "Gavi Summit Raises \$9B for Global Vaccines as U.S. Withdraws Support" (2025) — globalbiodefense.com
- Center for Global Development, "How Gavi 6.0 Can Take a Bigger Leap" (2025) — cgdev.org
- Lowy Institute, "US aid cuts are a soft power surrender to China" (2025) — lowyinstitute.org
- Council on Strategic Risks, "China Seizes an Opportunity from the United States in Humanitarian Aid and Disaster Response" (2025) — councilonstrategicrisks.org
- Jesse Marks (Substack), "Will China Fill the Gap Left Behind by Trump's Gutting of USAID?" (2025) — jessemarks.substack.com
- WUFT, "China and the U.S. alter foreign aid strategies" (2026) — wuft.org
- Devex, "How MacKenzie Scott quadrupled her philanthropic giving in 2025" (2025) — devex.com
- CNN / CBS News / AP, "MacKenzie Scott donated \$7.1 billion to nonprofits in 2025" (2025) — cnn.com; cbsnews.com
- Fortune, "MacKenzie Scott alone accounted for one-third of America's \$19.2B mega-gifts" (2026) — fortune.com
- Foreign Policy, "The Heyday of NGOs Ended Long Before the End of USAID" (2025) — foreignpolicy.com
- Global Policy Journal, "Cuts to USAID: fallout continues, Part 2" (2025) — globalpolicyjournal.com